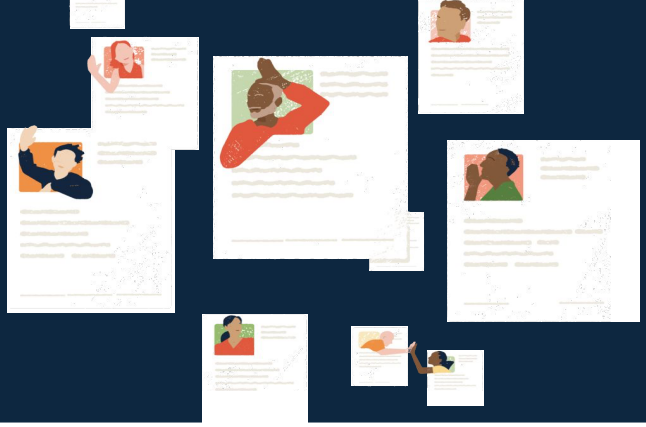
HealthEx

Unlocking Patient-Directed Data Exchange



Department of Health and Human Services (HHS); Centers for Medicare and Medicaid Services (CMS); Assistant Secretary for Technology and Policy (ASTP)
CMS-0042-NC; RIN 0938-AV68

# Request for Information Response: Health Technology Ecosystem

*Business Sensitive; this input is for government use only*

June 16, 2025

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# 1. EXECUTIVE SUMMARY

*See here for a brief video on HealthEx and its solution.*
*See here for an overview deck on HealthEx.*

HealthEx appreciates the opportunity to provide input on CMS and ASTP's Request for Information on the Health Technology Ecosystem. Our response reflects our commitment to ensuring that individuals can securely and effectively access, manage, and share their health information—grounded in a vision of patient empowerment and interoperable infrastructure.

**Who We Are**
HealthEx is a modern data rights management platform that enables secure, scalable, and compliant data access. Backed by General Catalyst and collaborating with major institutions such as MD Anderson, leading national payers, state HIEs, and ACOs, HealthEx has become a national leader in enabling Individual Access Services (IAS) under TEFCA. Our system is built with robust patient identity verification, digital consent, real-time data access and delivery, and endpoint discoverability.

**A Policy and Pilot Blueprint for CMS**
We propose a CMS-sponsored pilot project in which CMS partners with HealthEx to activate 100,000 Medicare beneficiaries to access and exchange their longitudinal clinical data across sites of care. This would enable CMS to close key care gaps that persist under claims-based infrastructure, understand the value of care provided, and alongside address fraud, waste, and abuse. Patients in turn are given the ability to access, retrieve, and direct access to their data to drive better care outcomes. The project would also highlight practical examples of issues patients run into while accessing their own data from the networks, and drive CMS-motivated change to resolve these.

The pilot leverages existing TEFCA IAS protocols—including IAL2/AAL2 digital identity verification and digitally enforceable consent—and will work in concert with CMS tools such as BlueButton 2.0 and MyMedicare.gov. By aligning policy with innovation, this pilot can serve as a springboard for CMS's digital health transformation goals, benefiting patients, providers, and plans alike.

**CMS Policy Recommendations from HealthEx**
In support of patient-directed exchange and the CMS vision for a modernized ecosystem, we submit six key policy recommendations (originally shared publicly via HealthEx in May 2025 as part of a policy memo):

1. Incentivize or mandate TEFCA participation for CMS-regulated entities to ensure IAS availability at scale.

2. Mandate responses to IAS requests and presume TEFCA-compliant verification meets HIPAA identity and authorization requirements.

3. Reject additional identity burdens, such as requiring patient portal credentials, when IAS requests use IAL2/AAL2 tokens plus consent.

4. Grant safe harbor to organizations performing patient matching as part of IAS fulfillment.

5. Mandate publication of provider and payer endpoints in a CMS-backed directory for record location services. We are pleased to hear CMS has made a commitment to support this type of endpoint directory.

6. Extend IAL2 identity token validity with refresh tokens (similar to Patient Access APIs), so patients don't need to repeat identity verification for each data access instance.

**These recommendations are aimed at removing friction from consumer-directed exchange and enabling patients to act as effective stewards of their data.**

**Please see here for a short video showcasing HealthEx and its capabilities and here for an overview deck on HealthEx.**

**RFI prepared by:**

- Priyanka Agarwal, MD, MBA, Co-founder and CEO at HealthEx
- Anand Raghavan, Co-founder and CTO at HealthEx

**Contributions from**:

- Melisa Soliz, Partner at Coopersmith Brockelman & HealthEx legal counsel
- Steven Lane, MD, MPH, Co-Chair of the Privacy & Consent Working Group for the Sequoia Project & HealthEx advisor
- Teresa Carlson, President of the General Catalyst Institute
- Candace Richardson, Principal at General Catalyst
- Hemant Taneja, CEO of General Catalyst

## 2. INTRODUCTION

HealthEx appreciates the opportunity to respond to CMS and the Assistant Secretary for Technology Policy's Request for Information on the health technology ecosystem. HealthEx's response is grounded in a shared vision: a healthcare system where patients are empowered to seamlessly access, control, and share their health information—securely, ethically, and in real time.

The stakes are high. Despite decades of investment in electronic health records and interoperability frameworks, individuals still struggle to access and direct their own medical information across care settings. This fragmentation harms patients, undermines equity, and impedes the shift to value-based, person-centered care. Yet the policy and infrastructure foundations now exist to change this fragmentation of data and the lack of patient control, especially with the support of aligned policy making from CMS.

**The HealthEx solution:**

HealthEx is a patient-driven data rights management platform for healthcare that enables fast, scalable and compliant access to the data that drives care. We are committed to building infrastructure that builds trust between patients and organizations around the use of their data, while giving patients access to their comprehensive data, across all their points of care. Its solution consists of:

1. **A consent and data access management platform** that allows organizations to efficiently collect, manage, and enforce patient consent and preferences, improving both patient trust and operational efficiency.
2. **A Patient-directed data exchange solution** to enable patient-directed data exchange, via IAS under TEFCA.
3. **An AI-driven data policy engine** that automates the enforcement of consent and other data policies, extending staff efficiency and data compliance.

*See here for a brief video on HealthEx and its solution, and here for an overview deck on HealthEx*

HealthEx's mission is to enable fast, scalable, and compliant access to health data, in a manner that is grounded in trust and transparency and done in partnership with patients and consumers. HealthEx is building the missing link: a secure, scalable, platform that enables patient-directed data exchange at national scale, while empowering consumers and patients.

HealthEx today is a company backed by General Catalyst, a leading venture capital firm. It is working with national leaders in healthcare, including MD Anderson Cancer Center, large (top 5) and regional payers, state Health Information Exchanges (HIEs), and large and regional accountable care organizations and independent care providers.

HealthEx has been a national leader in patient-driven data access[1] and interoperability: patient preferences, consent, control and transparency around data access are core to its services, as

[1] Recent Fierce Healthcare article on the MD Anderson / HealthEx strategic collaboration.

well as direct data retrieval from national networks, founded on individual access services (IAS) under TEFCA and patient-access APIs.

Our recommendations are designed to help CMS not only meet its goals for digital modernization, but to lead a transformation—where patients are no longer passive subjects in a system of closed data silos, but active participants in their care and partners in innovation.

We look forward to supporting CMS and its federal and private-sector collaborators in realizing this future.

**Please see here for a short video showcasing HealthEx and its capabilities, and here for an overview deck on HealthEx.**

## 3. BUSINESS INFORMATION

| HealthEx Business Information | |
|---|---|
| Company Name | HDX Labs, dba HealthEx |
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## 4. OUR UNDERSTANDING

CMS and ASTP seek to advance a patient-centered digital health ecosystem that is efficient, built on modern technical infrastructure, and with the appropriate guardrails in place to build trust and transparency. This includes expanding access to digital tools, reducing administrative burden, and ensuring that patients, providers, and plans have timely access to relevant health information to improve outcomes.

**From the medicare beneficiary's perspective, the current state of health data access is frustrating and fragmented.** Patients must log in to multiple portals, recall numerous credentials, and manually assemble records from disparate systems. Even where IAS is live, many consumers cannot yet access or exchange their data, because TEFCA participation is voluntary, and legal concerns about identity and authorization verification still limit responses to IAS requests. This makes patient-directed exchange inconsistent and inaccessible, especially for vulnerable populations.

Today, the infrastructure available to CMS, including Medicare Fee-For-Service and Medicaid programs, relies heavily on post-adjudicated claims and quality measures that can lag 12 to 18 months behind the actual patient experience. **This delay in access to data hinders CMS's ability to support real-time care coordination, accurate risk adjustment, timely identification of care gaps, and equitable benefit design.** Moreover, current systems often fail to deliver on the promise of interoperability for individuals—particularly those with chronic illnesses, fragmented care, or limited digital literacy.

HealthEx believes the infrastructure and policy foundations now exist to solve these challenges. HealthEx lays out the policy inputs that will enable a clear and secure framework for real-time, patient-initiated exchange, via IAS. The combination of IAL2-verified, AAL2-authenticated digital identity credentials, plus digitally enforceable and auditable IAS consent, provides a robust and HIPAA-compliant method to verify patient identity and intent.

Our understanding is that CMS's challenge is not technological feasibility—but operational readiness, policy alignment, and demonstration of value at scale. **CMS now has a historic opportunity to lead the cultural and technical shift toward patient-centered data exchange.** This can be done by aligning incentives, clarifying compliance expectations, and directly engaging patients as active participants in their care.

HealthEx is prepared to support CMS in this shift, both through concrete pilots and actionable policy recommendations. Our proposed pilot effort offers CMS real-time access to 100,000 Medicare beneficiaries' longitudinal clinical data. HealthEx can support CMS in reducing administrative burden, increasing digital equity, and achieving its interoperability goals—while ensuring patients themselves have the tools to manage and direct their own care.

## 5. BUSINESS OVERVIEW: HEALTHEX

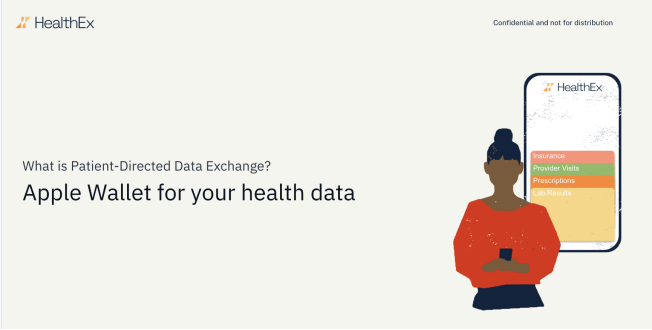
HealthEx is a modern data rights platform that enables fast, scalable, and compliant access to patient data, grounded in trust and transparency.

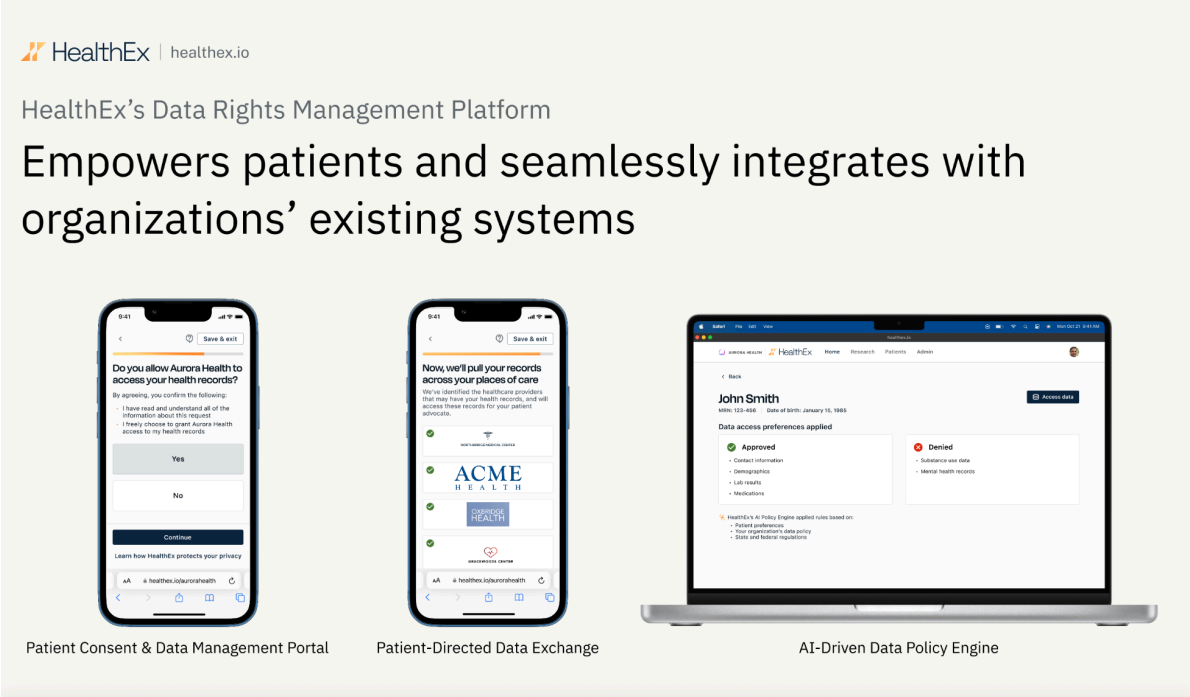
For teams looking to access, manage, or make patient data available to trusted third parties, HealthEx's platform offers:

| HealthEx Platform and Solution Overview | |
|---|---|
| *Solution Component* | *Description* |
| **A patient preferences and consent platform** | Collect patient consents around data access, and alongside give patients the means to view how their data is being used and change their mind, when needed. Consents are digitally enforceable & allow for granular data management (so patients can share portions of their clinical record). |
| **User-friendly interfaces for patient-directed data exchange** | Give individuals the power to access their health data across all their points of care, in one central location, and share it with any third party of their choosing – think "**Apple Wallet"** or "**Plaid"** for health data. |
| **An AI-driven data policy engine** | Review and automate the enforcement of consent and other data policies, at the points of data access, significantly reducing IT, compliance, and legal overhead, while empowering beneficiaries to permission their data for care, analytics, or research. |

**Please see here for a short video showcasing HealthEx and its capabilities, and here for an overview deck on HealthEx.**

*HealthEx enables payers real-time access to health data by leveraging patients' own right of access: We give individuals power to access their health data across all their points of care, in one central location, and share it with any third party of their choosing, via IAS under TEFCA.*
***Think "Apple Wallet" for health data: secure, portable, always in your hands.***






*HealthEx Platform Overview*

**Key Platform Differentiators, relative to industry competitors:**

| Key Platform Differentiators relative to Industry Competitors | |
|---|---|
| **User-friendly identity verification and authentication & no separate username/password required** | The platform leverages IAL2/OIDC tokens from well-known CSPs, or supports SMART-on-FHIR based authentication and app launches |
| **Ability to enable granular consent and segmented data access, for precision, and privacy, & patient control** | The platform has developed LLM-based data segmentation based on HL7 open standards for Value Set definitions. These allow classification of health data by resources or sensitivity scopes in |
| **Data retrieval that leverages accepted data standards, keeping data access costs low** | The platform leverages TEFCA's individual access services, Patient Access APIs, CMS Blue Button, CMS-0057F & 9115 APIs |

| | |
|---|---|
| **Lightweight deployment, with no separate app download required** | Lightweight web links integrates within member portals for patient engagement (no separate app download or username/password required) |

***HealthEx is one of the first teams enabling patient-directed data exchange nationwide. We are currently deploying our solution with leading national payers and state HIEs, and leading health systems like MD Anderson – additional announcements regarding these efforts will be coming shortly.***

### ROI and Key Value Drivers for Patients:

HealthEx's intent is to deliver value with every interaction to patients. Below are key benefits HealthEx provides to patients, by enabling patient-directed data exchange:

| Patient Benefit from HealthEx | Benefit Type |
|---|---|
| **Patients have access to their comprehensive records across all their places of care** - they no longer have to navigate disparate portals! | Empower patients with their clinical data |
| **Patients receive higher quality, more efficient care** since their payer can close care gaps, support care coordination across their providers, accelerate care claims & prior authorization approvals, and enable value-based care | Improved care journey |
| **Patients can use their data that's accessed via patient-directed data exchange to enable AI-based form filling**, to reduce redundant work for patients | Streamlined patient experience |
| **Payers can empower patients to earn a share of the economic value** their data generates by offering care-cost savings, cash rewards, or other value-based incentives when they consent to share their information. | Shared financial benefit |

### ROI and Key Value Drivers for organizations:

For payers and value-based care (VBC) organizations who today do not have real-time access to clinical data under the treatment exchange purpose, HealthEx is particularly meaningful and its real-time data access enables payers to:

| Payer Benefit & Value | How HealthEx Helps Payers |
|---|---|
| **Close care gaps** | Surface missing preventive screenings, labs, & follow-ups so providers can act faster, improve outcomes & boost risk-adjusted revenue. |
| **Meet quality incentive requirements** | Achieve CMS and payer program thresholds (e.g., Stars, HEDIS), unlocking bonus payments while avoiding penalties. |
| **Process prior authorizations or referrals more efficiently** | Automate data exchange and verification to shorten approval cycles, cut administrative spend, and speed patient access to care. |
| **Better understand the value of care delivered** | Link richer clinical and outcomes data to cost, enabling more accurate ROI analyses and stronger value-based contracting. |

**Company Experience & Leadership:**

HealthEx is working with leaders across US Healthcare, including top 5 payers, health systems that are internationally recognized in patient and cancer care, state HIEs, and a number of ACOs and VBCs.

HealthEx is a member of the CARIN Alliance - an industry collaboration focused on enabling consumer-directed data exchange, at scale. HealthEx is collaborative with these and other efforts, and to contribute our views to the national discourse.

HealthEx's leadership includes, Priyanka Agarwal, MD, MBA, co-founder and CEO at HealthEx, and Anand Raghavan, Co-founder and CTO. Dr. Agarwal is an internist by training and practiced medicine as an internist and hospital medicine attending for many years at UC San Francisco, where she is an Associate Professor of Medicine. She completed her residency in Internal Medicine at Brigham and Women's Hospital and UC San Francisco, obtained her MD and MBA from Harvard University, and completed her undergraduate studies at Stanford University. Mr. Raghavan was part of the founding team at BlueJeans Video Conferencing, acquired by Verizon in 2020, where he led the engineering and product teams. He was previously at Microsoft and Yahoo, and holds a degree in engineering from UC Berkeley.

**Privacy Commitments:**
HealthEx adheres to applicable federal and state privacy regulations and TEFCA privacy principles. We commit to the following:

- Data Minimization: We only collect & use EHI as necessary to fulfill authorized purposes.
- Purpose Limitation: EHI is accessed, used, or disclosed only for permitted TEFCA use cases.
- Consent Management: Patient-directed sharing is performed only after obtaining affirmative and informed consent, supported by documented policies.
- Data Subject Rights: Individuals can access, amend, or request deletion of their data as outlined in our Privacy Policy.
- Transparency: Our practices and responsibilities are communicated through policies, agreements, and user notifications.

Our full privacy policy can be found here: https://healthex.io/privacy-policy

**HealthEx Security Certifications:**
HealthEx's CSA Star Listing can be found here. HealthEx is also SOC2 and TX-Ramp certified. HealthEx implements security safeguards aligned with industry standards and SOC2 controls to protect EHI, including:

- Access Controls: Role-based access, least privilege, and multi-factor authentication (MFA) are required for all systems with EHI.
- Encryption: All EHI is encrypted in transit (TLS 1.2+) and at rest using AES-256 encryption via AWS KMS.
- Security Testing: Annual third-party penetration testing is conducted; most recent testing confirmed remediation of medium and low risk issues.
- System Monitoring: We continuously monitor for anomalies via infrastructure monitoring tools and intrusion detection systems (IDS).
- Incident Response: We maintain a formal incident response plan, including 24/7 on-call escalation procedures and post-incident review processes.
- Business Continuity: Disaster recovery plans are documented and tested annually.

HealthEx's internal security and compliance program is structured to meet or exceed:

- AICPA Trust Services Criteria (SOC2 – Security, Availability, Confidentiality)
- NIST SP 800-53a (moderate baseline, per Cacilian audit methodology)
- TEFCA security and privacy principles as defined in SOP-IAS

All employees undergo background checks, complete annual security awareness training, and sign confidentiality agreements.

**Government Contracting and Partnering:** HealthEx is also partnering with a government contractor who can supply HealthEx's services through their contract vehicles with CMS or associated GSA vehicles, as appropriate. HealthEx also has partnerships with companies that are providing contract vehicles to the government.

## 6. PILOT PROPOSAL: HEALTHEX / CMS

The pilot HealthEx is advocating for advances of CMS's goal to enable seniors and families to make informed health decisions with modern, trusted digital tools. The proposed pilot operationalizes the IAS exchange purpose under TEFCA—at national scale—to demonstrate patient empowerment, value, and real-time information flow, benefiting patients, CMS, and the entire health system.

### HealthEx / CMS Pilot Proposal

HealthEx to deliver 100,000 patient records within 90 days to patients and to CMS, via a pilot effort that provides CMS access to real-time records through patient-directed data exchange. Patients will alongside have access to their comprehensive records, to drive their care.

**Records will be made available, as below:**

- Patients will receive outreach from MyMedicare.gov, and will have the opportunity to access their comprehensive records, across all their points-of-care, and also make this data available to CMS.
    - Real-time records will be made available via patient-directed data exchange, via IAS under TEFCA, and falls back to available endpoint directories (potentially coordinated with CMS' own endpoint directory effort)
- Patient outreach, education and consent will occur via a joint HealthEx / MyMedicare.gov experience used by patients.
- Patients have the opportunity to opt-in to share their data.
- Upon consent, HealthEx accesses comprehensive patient records on the national networks, and delivers a copy of the records to the patient, and to Medicare, or the MA plan.

**Key Metrics and measures of success include:**

- Number of patients who opt-in to share their health records with CMS
- Volume of records made available to CMS
- Time to data access for CMS
- Patient satisfaction
- Number of care gaps closed for patients, degree of disease prevention / early diagnosis, ability to enable closed-roop referrals and smarter care coordination

**HealthEx Partners in this pilot effort:**

CLEAR

MedAllies

athenahealth
CURING COMPLEXITY

**These types of patient experiences showcase the art of the possible, and stand to deliver real benefit to patients and payers.**

# 7: RESPONSE TO RFI QUESTIONS

## 7.1 RESPONSE TO SECTION B: PATIENTS AND CAREGIVERS (PC-1 to PC-14)

**PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?**

**A. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?**
**B. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.**

**PC-1, A**.
**A patient's health data drives their care: providers, care facilities, payers, and the right teams having real-time access to health data is paramount to any individual managing their health.** With the challenges in US interoperability today, giving patients the control to access, retrieve, and direct access of their health data & record is 100% aligned with tech-enabled patient empowerment.

Today, the control over health data rests squarely with institutions: Organizations have controlled access to the health data that they create or receive. As a result, patients and individuals today face fragmented access to their records: They must log into multiple portals, with unique usernames and passwords, and piece together their health history manually, since organizations haven't given patients the means or the agency to combine or manage their data across institutions. This is especially burdensome for individuals with chronic conditions, or complex care needs, particularly those who see multiple providers.

We believe giving patients direct control over their data is more than a user-experience upgrade—it is a catalyst for systemic improvement. When individuals can easily retrieve and share verified health records, care teams gain timely, complete information that reduces duplicative testing, speeds diagnoses, and supports coordinated, value-based care. Patients who see real transparency and autonomy are also more likely to trust digital tools, grant consent for data use, and engage in preventive programs, ultimately lowering total cost of care while improving care quality and outcomes.

The advent of patient-directed data exchange, enabled via IAS, signals a shift from organizations controlling access to patient data, to now individuals having the power to do so (see Steven Lane's post that discusses the potential impact of giving patients more control over interoperability, and Hemant Taneja and Teresa Carlson's recent white paper on the role of patient-data access, in the context of broader healthcare innovation).

Patient-directed exchange streamlines the experience of how individuals access and share their health records; its key drivers of value for individuals are:

- **Convenience**: Individuals can request and receive their health records from across providers through a single, secure workflow.
- **Clarity**: Individuals gain a consolidated view of their health information—essential for coordinating care, managing conditions, and making informed decisions. There is also a growing opportunity to utilize AI or other apps to support individuals' understanding of their information and to provide actionable insights upon this consolidated view.
- **Control**: Individuals direct where their data goes, enabling them to authorize trusted third parties, like CMS, or caregivers, to use their data to provide better care. Individuals should also be able to selectively share their data and be able to easily revoke access to third parties.

When IAS functions as designed, it will enable patients to be active participants in their own care, not passive recipients: Individuals will have the power to direct data to new providers to allow them to easily gain a comprehensive view of their health history, to payers to support care coordination and process prior authorizations more efficiently, to research organizations enabling groundbreaking advances, or to any health app of their choice.

**PC-1, B**.

**If HealthEx were a personal assistant for patients, here are the top things we would help them do—enabled entirely through digital tools and modern infrastructure:**

1. **Find and access all your health records—no matter where you receive care**
   HealthEx allows individuals to connect to all of their past and current providers through a single, secure interface. Rather than logging into multiple portals or requesting records by fax, patients can retrieve comprehensive, longitudinal health data in just a few steps.

2. **Understand your medical history and what to do next**
   Once health records are accessed in real time, HealthEx helps patients (or their caregivers) make sense of them. This includes:
   - Summaries of diagnoses, medications, procedures, and lab results
   - Flags for missing information or fragmented history

- The ability to share records with a new doctor, family member, or digital health app

3. **Share your information securely with those you trust**
   Through HealthEx's digitally enforceable consent and consumer-directed data exchange service, patients can send their data to a provider preparing for a visit, to a caregiver supporting care navigation, or to a payer to streamline prior authorization. They can also revoke access at any time, with full transparency into how their data is used.

4. **Streamline your experience with AI-assisted form filling**
   HealthEx offers AI-powered tools that use the patient's retrieved data to automatically fill out common healthcare forms—such as intake paperwork, care coordination summaries, and eligibility verification forms. This not only reduces friction for patients and their caregivers, but also ensures greater accuracy by pulling directly from verified clinical data.

5. **Prepare for appointments, ask smarter questions, and follow up with confidence**
   Using HealthEx's real-time data access, patients and caregivers can generate visit prep summaries, identify recent changes in medication or labs, and ensure the provider has the full picture. This is especially valuable for patients with chronic conditions, multiple specialists, or recent transitions in care.

In short, the digital health products patients need most are tools that make data fluid, usable, and empowering. HealthEx meets this need by providing the underlying infrastructure that enables those experiences, while giving individuals autonomy, transparency, and trust in how their data is accessed and used.

By building upon TEFCA and enabling IAS at scale, HealthEx could serve as the "personal assistant" patients have long needed: one that helps them understand, manage, and act on their health information—on their own terms.

**PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?**

A. If so, what are some examples of benefits it has provided?
B. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?
C. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing

health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

**PC-2, A:**

When patients and caregivers can access their complete health records through a single platform or interface, the benefits are immediate and tangible:

- **Faster, more accurate care**: Providers gain access to a longitudinal record that includes diagnoses, labs, medications, and prior imaging—leading to faster diagnoses, more personalized treatment, and fewer duplicative tests.
- **Improved care coordination**: Especially for those managing multiple specialists or caring for loved ones, having a unified view supports better preparation for appointments and informed follow-up.
- **Reduced administrative burden**: Patients and caregivers can complete intake forms, care summaries, or medication lists quickly and accurately using pre-filled data.
- **Greater trust and engagement**: Individuals who see and understand their own data are more likely to engage in preventive care, ask questions, and participate in shared decision-making.
- **Empowered caregivers**: For aging parents, children, or family members with disabilities, consolidated access enables caregivers to support decision-making, medication management, and care transitions effectively.

These benefits are being realized today by users of the HealthEx platform, which enables real-time, patient-initiated data access across multiple care settings using TEFCA-aligned protocols and verified digital identity. However, this type of experience is still not available to most individuals today.

**PC-2, B:** For most Americans today, a single system that seamlessly aggregates health data across all providers still does not exist. This absence is most acutely felt in the following contexts:

- **Transition of care scenarios**: Patients discharged from hospitals or moving between providers need timely access to recent labs, imaging, and medication lists.
- **Caregiver support:** Family members often coordinate care for loved ones across states or health systems and need a central location to view, share, and act on records.
- **Chronic disease management:** Patients with complex conditions require data from multiple specialties that are rarely accessible in one place.

- **Pre-appointment preparation:** Patients want to arrive at appointments with relevant data in hand, reducing time spent recapping history and improving the quality of conversation.
- **Emergency care:** In urgent or out-of-network situations, access to an individual's complete history can be life-saving.

A unified Apple Wallet for health data— like HealthEx that supports these workflows—is essential to enabling a truly patient-centered healthcare system. Alongside, key to such improved data access and portability are the appropriate guardrails, which HealthEx builds in for individuals via tools that enable transparency around how their data is being used, and simple tools to change their mind regarding data access.

**PC-3. Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?**

**PC-3:** HealthEx's patient-directed data exchange solution, an Apple Wallet for health data, is a Medicare-ready, patient-directed data rights platform that lets beneficiaries and their caregivers assemble, control, and share a complete longitudinal record across every point of care. It pairs a plain-language, Web Content Accessibility Guideline-conformant interface with enterprise-grade security and compliance architecture so even first-time users can safely put their data to work.

Key components and why they matter:

1. **Consent & data-management portal:** Beneficiaries set granular sharing rules—"my cardiology notes, not my mental-health records"—through step-by-step prompts, in-context education, and optional caregiver delegation. All preferences are digitally enforceable and fully auditable, protecting privacy while saving staff from manual consent tracking.
2. **Patient-directed exchange services:** Using TEFCA-approved Individual Access Services, the wallet retrieves verified records across national networks in near-real time. Members can then share a one-time or ongoing data link with any trusted third party, streamlining second opinions, remote monitoring, or Social Security disability reviews.
3. **Enterprise data-rights management engine:** Before any record is released, automated checks confirm that the request aligns with CMS, HIPAA, and organizational policy, shortening IT and compliance review cycles from weeks to seconds and preventing accidental disclosures.

HealthEx is healthcare native, AI native, and patient centered. By combining intuitive patient controls with rigorous security, HealthEx turns fragmented data into a usable asset for better navigation, faster authorizations, and more coordinated, value-based care.

**PC-4. What features are missing from apps you use or that you are aware of today?**
**a. What apps should exist but do not yet? Why do you believe they do not exist yet?**
**b. What set of workflows do you believe CMS is uniquely positioned to offer?**

**PC-4:** Apple Health offers a focused set of functionality to allow patients to securely collect and store wearable data and aggregated health data from a limited set of providers who Apple lists as partners. While admirable in its commitment to privacy and security, the product as available today cannot be meaningfully used to simplify or accelerate interactions with the various points of care most patients get in the real-world - such as pharmacies, doctor visits, etc. - and adoption is limited. To the extent sharing of this data is possible, it is controlled via HealthKit and the Apple App store ecosystem. This promotes a closed ecosystem of a user's Apple-only devices and Apple-approved apps managing access to your health data, whereas it is in the healthcare sector's best interest to ensure the patient always has additional options to move their data in an open, cross-platform manner, including on the Open web for webapps, AI agents etc. The ideal outcome is the best of both worlds: an Apple-like sleek consumer experience to manage and get value out of one's health data that interoperates with other standards-based workflows and the open internet.

B. The biggest roles CMS is uniquely positioned to play are to educate the average American re: their health data rights, and ways in which they can apply these towards everyday improvements in their care journeys, and to ensure providers and payers nationwide are held accountable to release data to patients when requested compliantly through the networks.

**PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?**

**B. What changes would enable timely access to high quality CMS and provider generated data on patients?**

**PC-5, B:**

Key policy inputs requested from CMS to enable consumer-directed data exchange, at scale, and in turn allow patients to share their data to CMS, in real-time:

| | CMS Policy inputs to support patient-mediated data exchange | |
|---|---|---|
| # | CMS Policy Input | Rationale |
| 1 | **CMS should Incentivize or Mandate TEFCA participation** | While responding to IAS queries is presently required of all TEFCA participants, participation in TEFCA is currently voluntary. To ensure that IAS is supported at scale, it is necessary for CMS to mandate that CMS-regulated health plans & health care providers participate in TEFCA, or create meaningful financial incentives for doing so. Such CMS support will inevitably drive other payers to follow suit. |
| 2 | **CMS should mandate responses to IAS requests and recognize TEFCA protocols as sufficient to satisfy HIPAA privacy requirements** | Not all TEFCA Participants and Subparticipants are currently responding to TEFCA IAS requests due to concerns that legal preconditions regarding verification of identity and authority are not being met, potentially opening up responders to such queries to an increased risk of a privacy breach. TEFCA has specified technical security measures to support such verification: A single IAL2 verified, AAL2 authenticated digital identity token satisfies identity verification and authentication requirements, and an additional IAS consent administered to the patient, that is auditable and digitally enforceable, meets all applicable authorization verification requirements.<br><br>A CMS mandate requiring responses to IAS requests made via TEFCA, with an express presumption that such verification requirements are met when the request satisfies TEFCA standards, would ensure patients and consumers have secure access to their health data.<br><br>An IAS service that doesn't return responses will be meaningless to patients and consumers and discourage use and adoption. |
| 3 | **CMS should adopt a presumption that IAS requests made through TEFCA adequately fulfill identity verification and authorization requirements to facilitate patient matching for an individual accessing their own data,** | Some plans, providers and their business associates, such as their EHR vendors (e.g., Epic, Oracle/Cerner), encourage or require an additional verification step where patients must provide their portal logon credentials for each provider organization from which they want to request access to their health information. This added step may be required by the data holding organization in their efforts to assure satisfaction of HIPAA's identity and authority verification requirements. However, |

| | CMS Policy inputs to support patient-mediated data exchange | |
|---|---|---|
| # | CMS Policy Input | Rationale |
| | **without additional requirements for individuals to provide their patient portal credentials.** | this added burden is not always required by other data holders, such as the users of Athena Health and Veradigm EHRs.<br><br>These additional steps may and should be removed when patients are trying to access their own data as both verification and authorization requirements can be met by the IAL2 token + AAL2 authentication + IAS consent passed in via TEFCA IAS.<br><br>With these changes, IAS flows will be more user friendly, ensuring that all individuals, at varying levels of health and tech-based literacy will be able to engage with IAS and be able to use their health data to its fullest capacity. |
| 4 | **Grant qualified immunity, or safe harbor, to those individuals and organizations performing patient matching as part of responding to IAL2-enabled IAS requests** | Granting qualified immunity, or safe harbor, to those responding to IAS requests, and other permitted exchange purposes via TEFCA, will reduce the hesitancy of health plans, providers, and other TEFCA participants/sub-participants to respond to defined use cases with a published SOP, beyond the Treatment use case. |
| 5 | **Mandate that providers publish and maintain FHIR API endpoints that will respond to IAS queries, for inclusion in a CMS-mandated national endpoint directory that can be repurposed by various Record Locator services. (This could be as simple as making existing TEFCA and Carequality RLS directories public.)** | Rationale: CMS should mandate the publication and maintenance of a comprehensive endpoint directory that allows accurate record location for patients, to support IAS requests at scale. Without this discoverability to facilitate accurate record location, patients cannot obtain access to records across all their places of care.<br><br>**UPDATE**: We are pleased to note the announcements made by CMS on this front and the commitment expressed towards building a national healthcare endpoint directory. We believe this is a very positive step in the right direction. |
| 6 | **CMS should mandate that the IAL2-assured OIDC token persist for an extended period, up to 12** | At HealthEx, we celebrate the possibilities unlocked for patients via TEFCA's IAS purpose. The current IAS SOP deems the IAL2-assured OIDC token provided by a Kantara initiative-certified CSP as good enough to be presented as |

| | CMS Policy inputs to support patient-mediated data exchange | |
|---|---|---|
| # | CMS Policy Input | Rationale |
| | **months, to enable patients to direct their data access to occur in a recurring, ongoing fashion, for use cases under IAS.** | authentication and authorization to QHIN's to access one's own data. Unless gated further by Facilitated FHIR endpoints, as some vendors have continued to do, this represents a big win in user experience for patients trying to aggregate their records for patient-directed exchange or release to a 3rd party. It also represents a significant improvement over the previous Patient Access APIs workflow of entering credentials from portal to portal.<br><br>This satisfies a one-and-done use case. However, patients typically agree to make their data flow to trusted 3rd parties for longer periods of time - weeks, months, maybe even a year - to avoid the headache of having to re-login constantly. This used to be addressable in patient portal-based SMART-on-FHIR workflows by the requesting application informing a patient about the duration of data access they were requesting, operationalized via a refresh token for the desired period of time upon login. Patients also had the ability to revoke their consent at any time.<br><br>However, in IAS, the OIDC tokens typically expire within an hour (often 5-15 mins), after which any data access on behalf of the patient has to be reauthenticated biometrically. The concept of refresh tokens does not exist. While we can acknowledge the security of this approach, this represents a significant enough degradation of user experience for patients over the prior portal-based Patient Access APIs that TEFCA IAS is otherwise trying to improve upon that we believe is an oversight that has to be fixed. |

**PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?**

To make digital health products truly accessible for Medicare beneficiaries and their caregivers, especially those new to technology, three feature design principles matter most:

1. **Immediate, obvious value**: The product must solve a real problem the first time it is used—e.g., retrieving a medication list or specialist note in seconds. Clear benefit builds trust and drives repeat use.
2. **Radically simple experience**: Meeting users where they already are - embedded in their Medicare.gov portal experience, or in existing patient portals - without need for new app downloads, authentication or accounts is critical in our opinion. Large fonts, plain-language prompts, other accessibility features and a linear "one-task-per-screen" flow additionally reduce cognitive load.
3. **Consent-driven data control**: An Apple wallet-style interface that lets beneficiaries (or authorized caregivers) retrieve, store, and share verified health records with a single tap empowers them to orchestrate their own care. Granular permissions keep privacy intact, and familiar identity-verification methods (e.g., driver-license scan + selfie) make onboarding seamless.

**PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?**

**PC-7:** HealthEx strongly supports CMS's interest in measuring the real-world impact of digital health tools. To ensure this measurement is effective, scalable, and equitable, CMS should anchor its data collection strategy around three core principles: (1) patient-directed data access, (2) standardized interoperability, and (3) embedded feedback loops within real-world care settings.

1. **Leverage Individual Access Services (IAS) to Enable Longitudinal Patient-Centered Data Collection**

   HealthEx recommends CMS leverage patient-directed data access, via IAS, to obtain real-world data to quantifiably measure care impact resulting from digital health products. By supporting a CMS-sponsored pilot in which 100,000 Medicare beneficiaries access and manage their own data using HealthEx's patient-directed data exchange service, CMS can observe the full clinical trajectory of patients, and collect insights that would not be available via claims alone

   This approach is privacy-protective, standards-based, and fully auditable—ensuring real-world data is trustworthy and longitudinal.

2. **Use FHIR APIs and Digital Identity Credentials to Minimize Friction**

   HealthEx's platform integrates **FHIR APIs, SMART on FHIR protocols, and IAL2/AAL2 digital identity credentials**, enabling seamless, verified access to clinical data across systems. CMS should encourage digital health vendors participating in pilot programs or payment models to expose FHIR endpoints, support patient-directed queries, and register in a national directory. This infrastructure enables near real-time outcome tracking and reduces reliance on retrospective claims analysis.

3. **Embed Evaluation in Care Workflows and Pair It With Digital Consent**

   HealthEx recommends CMS integrate HealthEx's **digitally-enforceable consent capture** into its data strategy. This ensures that patients explicitly authorize the use of their data for research and evaluation, while allowing CMS to gather contextual information—such as patient experience, product usage patterns, or disparities in impact—that may not be visible from health records alone.

   CMS can also collaborate with providers, researchers, and value-based care organizations to embed HealthEx-enabled data sharing into existing workflows (e.g., chronic disease management, hospital-at-home programs) and evaluate outcome changes in situ.

**PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?**

**A. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?**
**B. What are specific sources, other than claims and clinical data, that would be of highest value, and why?**
**C. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?**

**PC-8 A**. Clinical data in consistent formats that meets a minimum quality standard is still hard to obtain across the long tail of providers operational across the country today.

**PC-8 B.** Making Imaging data, particularly those from PACS servers sitting alongside standard EHRs, in lossless fashion would be a big benefit for app developers and AI agents.

**PC-8 C**. Establishing a common quality standard and rubric for all data shared by different sources that can be listed in the national directory as a property of each source. We recommend taking a look at more novel, creative approaches to doing this as opposed to an overly constrained structured standard - using an open-source LLM-as-a-judge for example, and making all relevant weights public for transparency.

**PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?**

**A. What difficulties are there in accessing or utilizing these data sources today?**
**B. What suggestions do you have to improve the Blue Button 2.0 API experience?**
**C. Is there non-CMS data that should be included in the API?**

We advocate for CMS leadership in establishing a nation-wide 'BlueButton 3.0' concept that centers on clinical data exchange (replicating what Blue Button 2.0 + Data at the point of care did for Claims data for Medicare beneficiaries), with the right consent semantics and enforcement. We want this to be established by CMS as a blueprint for TEFCA IAS at scale - either by embracing TEFCA's IAL2+IAS themselves, or by first implementing this solution for Medicare beneficiaries, then requiring CMS-reimbursed providers to support this for all their patients.

While the data feeds established under Blue Button 2.0 provide value, claims and demographic data are not sufficient for driving value for the average medically complex patient managing a complex diagnosis - claims data will typically be available 30-90 days after an episode of care, at which point its utility is limited.

**PC-10. How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world? Please provide specific examples.**

**What changes would you suggest?**

**What use cases could have a significant impact if implemented through TEFCA?**

**What standards are you aware of that are currently working well to advance access and existing exchange purposes?**

**What standards are you aware of that are not currently in wide use, but could improve data access and integration?**

**Are there redundant standards, protocols, or channels that should be consolidated?**

**Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?**

**PC10, A**. IAS under TEFCA is incredibly meaningful, as we've outlined above, and is not available or supported via other national data access networks.

**PC10, B**. Missing from TEFCA today is a standardized consent framework and service that is auditable, digitally enforceable, and granular in nature. HealthEx terms this 'computable consent,' and has been actively engaged in building this out with several national standards groups. Unlike Identity, which is well defined and supported by CSP certification by 3rd parties such as the Kantara Initiative, consent remains relatively loosely defined, implemented and enforced.

HealthEx's computable consent concept envisions a neutral, trusted service (ie one not owned by any single traditional health system/payor/pharma player) that records patient consent across categories of healthcare data use cases in a way that is deeply tied with their digital identity and can be repurposed across healthcare organizations in categories of contexts. In this way, it acts like a personal assistant or agent for the patient to help them navigate the legal complexity of consents and other documents across their care journeys. Below is additional information on HealthEx's consent solution:

**Key features of HealthEx's Computable Consent Platform:**

- A highly interactive system of presenting specialized templates for gathering informed consent based on the nature of research driving the request (using rich media, integrated with medical professional inputs etc.)
- Biometric (IAL2) or SMART-on-FHIR based authentication for users providing consent
- A system of recording consent for the patients' rights to be contacted/forgotten

- A system of recording patient consent per data access type on a ledger in an immutable, time-stamped, and securely encrypted manner that minimizes risk of disclosure
- An audit mechanism to ensure screen-by-screen replay of the workflow a patient went through at the time of signing their consents
- A system that allows for efficient lookups by applications or AI agents to ensure recorded consent is honored and enforced by policy engines and governance teams, while releasing or delivering data in real-time
- A system of protocol amendments for health systems/pharma and re-consenting/updating consent choices for patients
- Demonstrable compliance with informed consent documentation expected across languages and regulatory environments - HIPAA etc
- A patient-facing component that acts as a Patient Consent Manager (similar to a password manager)
- Integration/embed/API/Data warehouse libraries for Health system data requests to check for necessary consent checks before sharing the data

Beyond consent, Payers are under-represented and underserved by TEFCA and networks such as CareQuality. Establishing a real Operations purpose inclusive of all providers and payers (with public endpoint directories to go with it) unlocks tremendous value for all involved.

**PC10, C. HealthEx strongly believes that IAS represents the most transformative use case under TEFCA**, and that CMS is at a critical inflection point to operationalize it at scale. By treating the patient as a first-class data actor, IAS unlocks a new class of digital health services that are both person-centered and system-enabling.

We propose the following high-impact **use cases for implementation through IAS:**

1. **Patient-Directed Data Exchange for Care Coordination and Transitions**

   **Impact:** Patients retrieving and sharing their data—via IAS—with new providers during referrals, hospital discharges, or care transitions.

2. **Patient-Enabled Prior Authorization Support**

   **Impact:** Patients use TEFCA IAS to retrieve recent clinical data (e.g., imaging, labs, notes) and share it with payers to expedite prior authorization decisions.

3. **Pre-Visit Record Retrieval for Specialty Referrals**

   **Impact:** Patients aggregate records from multiple providers and send them to a specialist in advance of their visit, using TEFCA IAS.

4. **AI-Assisted Form Filling and Social Services Access**

**Impact:** Using data retrieved through TEFCA IAS, HealthEx supports beneficiaries in **automatically populating forms** for services like SNAP, Medicaid redetermination, transportation, or home health—augmented by AI.

5. **Caregiver-Enabled Proxy Access**

   **Impact:** Family caregivers or legally authorized representatives use IAS on behalf of patients—retrieving and sharing records to coordinate care, manage medications, and engage with providers.

6. **Real-World Data Contribution for CMS Evaluation**

   **Impact:** Patients grant CMS permission to retrieve their TEFCA-accessible clinical data to support program evaluation, research, and quality improvement. Voluntary, patient-mediated data sharing offers CMS a scalable way to gather real-world evidence about outcomes, disparities, and digital health adoption—without placing additional reporting burden on providers.

By promoting these IAS-centered use cases, CMS can transform TEFCA from a compliance framework into a catalyst for better outcomes, greater efficiency, and true consumer empowerment in healthcare.

HealthEx stands ready to support CMS in piloting, scaling, and evaluating these use cases—demonstrating their impact in real-world settings across diverse beneficiary populations.

**PC10, E**: HealthEx is partnering with several consent frameworks across the ecosystem. Its computable consent model is consistent with the FHIR at Scale Taskforce (FAST) model, and with themes laid out in the Sequoia Group's recent whitepaper "Moving Toward Computable Consent: A Landscape Review" (https://sequoiaproject.org/wp-content/uploads/2025/01/Sequoia-Consent-Landscape-Review-for-Public-Comment-012325.pdf)

The HL7 FHIR at Scale Taskforce has issued a paper entitled "Consent at Scale" (https://confluence.hl7.org/display/FAST/Consent+Management+Supporting+Materials?preview=/204279212/204279403/FAST-consent-at-scale-2023-08-20-final-version.pdf). The paper is overall quite excellent and considers a number of abstract concepts that are highly relevant and relatable to HealthEx. HealthEx's consent model aligns with nearly all the requirements laid out by this taskforce and whitepaper, with a gap analysis available upon request.

**PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?**

**PC-11:** As patients' mobility and digital interactions around their health increase, the existing state-based HIE models alone are not going to be sufficient to scale to break silos and provide a unified user experience for patients, app developers and other ecosystem players. There are numerous examples where patients cross state boundaries to get care, or relocate regularly for seasonal employment or other reasons, or are seeking assistance from remote care providers unconnected to their geographical location. TEFCA or equivalent national directories/exchanges are better placed to act as a clearinghouse in these scenarios.

However, there may be a lot of untapped potential in HIEs that have successful and well utilized direct patient interfaces to support propagation of digital identity and computable consent models within existing workflows.

## 7.2 RESPONSE TO SECTION B: PROVIDERS (PR-1 to PR-14)

**PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?**

Please see our policy recommendation regarding safe harbour in the patients and caregivers section above.

**PR-11. How could members of trust communities (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?**

**PR-11:** The first important step is for all the QHINs to come together and agree to fully implement the latest version of the IAS SOP consistently among themselves for true IAS interoperability. At this point, we are aware of only 1or 2 (ex. MedAllies) that have already done this, while other QHINs may be looking for OIDC token attributes slightly different from those currently being issued by IAL2 vendors like CLEAR in production. Related to this, every QHIN needs to publish a set of test patients that other QHINs and developers can use towards testing their interoperable connectivity and ability to receive data.

The second important step is for the QHINs to implement a mechanism to allow for multiple lookups on some periodicity or on demand for the same user without requiring reauthentication (ex exchanging refresh tokens for access tokens), based on the initial IAL2 verification. This will greatly reduce the burden on the patient who often doesn't want to be contacted every time their updated data would be required.

## 7.3 RESPONSE TO SECTION D: PAYERS (PA-1 to PA-7)

**PA-1. What are the policy, programmatic, and technical limitations to TEFCA participation or use that affect payers' ability to effectively and efficiently exchange data?**

**a. What changes could CMS make to enable greater payer participation and data exchange in TEFCA?**

**PA-1 A**: Require that CMS-regulated health plans and health care providers respond to certain Healthcare Operations sub-exchange purposes, such as use cases for care coordination / case management, HEDIS quality measure reporting, risk adjustment, and monitoring for potential fraud, waste, and abuse.

Treatment is currently the only purposes for nationwide health data exchange that have been adopted at scale by the national data sharing frameworks, such as Carequality and TEFCA; IAS is an approved use case with limited adoption. Although a framework for the operational use case is loosely defined, it has not yet seen widespread adoption or acceptance. TEFCA has developed an SOP that would advance specific operations use cases, and we encourage CMS to use its policy levers to also make these mandatory and expand them to include fraud, waste, and abuse (FWA), and risk adjustment. Of note, FWA as a use case is distinct from HIPAA-permitted operation exchange purposes supported by TEFCA today. To advance the goals of finding and ending fraud, waste, and abuse it would be necessary to require the return of data for that purpose.

**PA-3. What are opportunities and limitations for the use of digital identity credentials (for example, CLEAR, ID.me, Login.gov) to simplify patient, provider, and plan identity verification by payers?**

See answer to PA-1. Long-lived tokens equivalent to the refresh tokens issued by EHRs/patient portals today during patient access API based lookup would greatly help adoption.

**PA-4. What would be the benefits and burdens of requiring all CMS-regulated payers to enable public access to their provider directories in a standardized FHIR-based format?**

This would be a good way to bootstrap a national endpoint directory. Please ensure NPIs of providers are exposed as well as logos of the providers (many patients recognize their providers based on logo, rather than textual names).

## 7.4. RESPONSE TO SECTION F: VALUE-BASED CARE ORGANIZATIONS (VB-1 to VB-15)

**VB-2. What are the most important technical capabilities CMS could promote to support the needs of organizations in value-based care arrangements?**

**a. Examples may include: data aggregation and normalization, real-time ADT notifications, eCQM measurement support, beneficiary/patient-facing tools for navigation, data capture, and outcomes reporting.**

**VB-2:** To successfully participate in value-based care (VBC) arrangements, organizations must be able to access comprehensive, timely, and actionable health data across disparate care settings. Yet today, technical fragmentation, data latency, and information silos undermine the goals of coordinated, outcomes-driven care.

HealthEx recommends that CMS prioritize the following **technical capabilities**, all of which are foundational for enabling VBC organizations to deliver high-quality, patient-centered care and meet performance benchmarks:

**1. Patient-Directed, Real-Time Data Access via IAS under TEFCA**

Value-based care depends on complete visibility into a patient's longitudinal health journey—including encounters outside of a given ACO, health system, or payer network. HealthEx's platform leverages TEFCA IAS to enable patients to retrieve and share their health records from any QHIN-connected provider, using IAL2/AAL2 digital identity and digitally auditable consent.

CMS should promote this infrastructure as a way for providers and VBC entities to:

- **Receive patient-authorized records in real time**
- **Access data from unaffiliated providers and facilities**
- **Support care transitions and reduce duplication**

IAS-based access ensures that beneficiaries are true partners in managing their care, while giving VBC organizations the clinical intelligence they need—regardless of EHR vendor or institutional affiliation.

**2. Interoperable APIs for Automated, Standards-Based Data Capture**

VBC organizations struggle to normalize and ingest data from heterogeneous sources. CMS should continue to advance the adoption of:

- **FHIR APIs for patient and population-level data**
- **SMART on FHIR protocols for modular integration with apps**
- **Bulk FHIR for performance measurement and population health analytics**

HealthEx already supports these standards and offers tooling that enables data normalization, deduplication, and quality filtering—making it easier for downstream systems to compute eCQMs, analyze utilization patterns, and manage risk-based contracts.

**3. Digital Identity Infrastructure to Streamline Cross-Entity Access**

Interoperability is limited when every entity maintains its own login credentials and access pathways. CMS can reduce administrative burden and expand patient access by promoting adoption of:

- **NIST 800-63-3 IAL2/AAL2 digital identity credentials (e.g. CLEAR, ID.me)**
- **Federated identity models that are portable across VBC entities**

HealthEx integrates identity credentials into its access and consent flows, enabling patients to interact with multiple VBC participants through a single, secure identity—facilitating proxy access, caregiver delegation, and coordination across providers.

**4. Patient-Facing Tools to Surface Gaps and Capture Outcomes**

To succeed in VBC, organizations need tools that:

- Empower patients to view and act on their health data
- Auto-fill forms and assessments with structured clinical data
- Capture patient-reported outcomes and social needs

HealthEx supports these capabilities by enabling individuals to:

- Retrieve records from multiple systems
- Generate a consolidated longitudinal view
- Use AI-powered form filling for care navigation and social services access
- Share data with authorized care teams or support services

CMS should consider incentivizing digital health platforms that deliver these patient-empowerment tools—especially those that are accessible, multilingual, and inclusive of underserved populations.

The technical infrastructure that underpins successful value-based care must be **interoperable, patient-directed, standards-aligned, and real-time**. HealthEx's platform operationalizes these principles today, and we stand ready to support CMS in expanding access to these capabilities across the care continuum.

**VB-3. What are essential health IT capabilities for value-based care arrangements?**

**a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, benchmarking against peers, patient navigation tools, predictive analytics, workflow optimization.**

A cornerstone of VBC is the ability to act on **timely, complete, and continuously refreshed patient data**. Care teams need access not only to a patient's historical record but to a **real-time stream of new health information** across providers and care settings captured either periodically or in an event-driven manner. Yet today, this foundational requirement—**recurring, real-time access to health data**—is not reliably possible due to lack of a scalably implemented Operations/Payments purpose use case on TEFCA and other national networks, and operational as well as incentive alignment issues involved in negotiating Provider-to-Payer release of sufficiently detailed clinical data.

This gap imposes a ceiling on VBC effectiveness. Capabilities like predictive analytics, longitudinal care planning, event notifications, and proactive gap closure all depend on **repeated data access**, not just one-time retrievals. Without a reliable way to maintain real-time connections to the patient record, even the best VBC tools are forced to work with outdated or partial data.

**HealthEx's Perspective: Unlocking Persistent, Patient-Directed Data Access**

HealthEx addresses this challenge by leveraging **TEFCA Individual Access Services (IAS)** and Patient Access APIs exposed by Providers, combined with **IAL2/AAL2 digital identity credentials** and **digitally enforceable consent**. This combination allows a patient to:

- Retrieve their own longitudinal health data from QHIN or other national endpoint directories' connected sources

- Authorize trusted parties (e.g., a VBC organization) to **receive recurring updates** with minimal friction (esp if data is made available in brokered fashion through the networks)

By enabling patients to serve as **ongoing data stewards**, with the ability to monitor how their data is used and change their mind at any time, HealthEx turns the current institution-by-institution data source model into a **secure pipeline of on-demand or periodic real-time clinical updates** mediated by patients.

A key limitation slowing us down is the short lived OIDC token validity period issued by CSPs today with no option for a refresh token provided by the networks. This results in the user having to potentially reauthenticate themselves for nth-time data updates/pulls.

**Key Capabilities Unlocked by Recurring, Real-Time Access**

With this foundation in place, a wide range of VBC-enabling functions become operational and impactful:

- **Care planning:** Providers can access the latest encounters, labs, and imaging across care settings.
- **Patient event notification:** Updates like hospital admissions or ED discharges can be retrieved by patients and shared with care managers or home health teams.
- **Data normalization and quality measurement:** HealthEx aggregates and standardizes records across EHRs to support eCQMs, benchmarking, and peer comparisons.
- **Predictive analytics and navigation:** AI tools perform better when fed complete, fresh data—guiding patients through follow-ups, preventive screenings, and social supports.
- **Workflow optimization:** Staff burden is reduced when data can be retrieved once by the patient and reused across referrals, authorizations, and care transitions.
- **Quality Metrics measurement** for meeting incentive requirements.

**VB-15. How would a nationwide FHIR-based provider directory and/or patient access API directory benefit your organization's participation in value-based care arrangements?**

This would be a really strong way to bootstrap a national endpoint directory that can be reliably used by patients and developers alike. Please ensure NPIs of providers are exposed as well as logos of the providers (many patients recognize their providers based on logo rather than textual names). HealthEx would switch to using such a directory as its primary lookup option if it enforced format consistency, and supported the asks above.